

Booked For
Gurugram, Pathkind Diagnostics Pvt. Ltd., City Court
Ground Floor Sector 26 MG Road, P.O.DLF Phase 1,
Gurugram - 122002
Contact No. - 8929929919

Processed By
Gurugram, Pathkind Diagnostics Pvt. Ltd., City Court Ground
Floor Sector 26 MG Road, P.O.
DLF Phase 1, Gurugram - 122002
Contact No. - 8929929919

Name : Mr. SOURAV YADAV
Age/Gender : 28 Yrs/Male
P. ID No. : 1800D009202650530
Accession No : **1800C00920265050030**
Referring Doctor :
Referred By : Self

Billing Date : 05/05/2026 09:06:47 AM
Sample Collected on : 05/05/2026 09:06:50 AM
Sample Received on : 05/05/2026 11:27:21 AM
Report Released on : 07/05/2026 02:54:27 PM

Report Status -Final

Test Name	Result	Biological Ref. Interval	Unit
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FEVER PANEL- 2

BIOCHEMISTRY

SGPT / ALT 6.24 0.00 - 41.00 U/L
Sample : Serum
Method : Spectrophotometry

CLINICAL PATHOLOGY

Urine Routine & Microscopic Examination

Sample : Urine, Random

Colour, Urine <i>Method : Manual</i>	Straw	Pale Yellow	---
Appearance <i>Method : Manual</i>	Hazy	Clear	---
Specific Gravity <i>Method : Ionic concentration method</i>	1.025	1.00 - 1.03	---
pH <i>Method : Double indicator principle</i>	6.00	4.70 - 7.50	---
Glucose <i>Method : Benedict's Method</i>	Not Detected	Not Detected	---
Protein <i>Method : Sulphosalicylic acid Method</i>	Not Detected	Not Detected	---
Ketones <i>Method : Rothera's Method</i>	Not Detected	Not Detected	---
Blood <i>Method : Microscopy</i>	Not Detected	Not Detected	---



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Bilirubin <i>Method</i> : Fouchets Method	Not Detected	Not Detected	---
Urobilinogen <i>Method</i> : Ehrlich's Reaction	Normal	Normal	---
Nitrite <i>Method</i> : Nitrite Test	Not Detected	Not Detected	---
Pus Cells <i>Method</i> : Microscopy	8-10	0-5	/hpf
RBC <i>Method</i> : Microscopy	Not Detected	Not Detected	/hpf
Epithelial Cells <i>Method</i> : Microscopy	5-7	0-5	/hpf
Casts <i>Method</i> : Microscopy	Not Detected	Not Detected	---
Crystals <i>Method</i> : Microscopy	Not Detected	Not Detected	---
Bacteria <i>Method</i> : Microscopy	Not Detected	Not Detected	---
Remarks <i>Method</i> : Manual	Microscopic examination has been performed on urine sediment.		

HAEMATOLOGY

Complete Blood Count (CBC)

Sample : Whole Blood, EDTA

Haemoglobin (Hb) <i>Method</i> : Photometric	15.30	13.00 - 17.00	gm/dL
Total WBC Count / TLC	7.03	4.00 - 10.00	thou/ μ L



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<i>Method : Impedance</i>			
RBC Count	4.85	4.50 - 5.50	million/ μ L
<i>Method : Impedance</i>			
PCV / Hematocrit	44.20	40.00 - 50.00	%
<i>Method : Impedance</i>			
MCV	91.20	83.00 - 101.00	fL
<i>Method : Calculated</i>			
MCH	31.60	27.00 - 32.00	pg
<i>Method : Calculated</i>			
MCHC	34.70 H	31.50 - 34.50	gm/dL
<i>Method : Calculated</i>			
RDW (Red Cell Distribution Width)	12.00	11.80 - 15.60	%
<i>Method : Calculated</i>			
Neutrophils	73.00	40.00 - 80.00	%
<i>Method : Impedance</i>			
Lymphocytes	23.00	20.00 - 40.00	%
<i>Method : Impedance</i>			
Eosinophils	2.00	1.00 - 6.00	%
<i>Method : Impedance</i>			
Monocytes	2.00	2.00 - 10.00	%
<i>Method : Impedance</i>			
Basophils	0.00	0.00 - 2.00	%
<i>Method : Impedance</i>			
Absolute Neutrophil Count (ANC)	5131.90	2000.00 - 7000.00	/ μ L
<i>Method : Calculated</i>			
Absolute Lymphocyte Count	1616.90	1000.00 - 3000.00	/ μ L
<i>Method : Calculated</i>			
Absolute Eosinophil Count (AEC)	140.60	20.00 - 500.00	/ μ L
<i>Method : Calculated</i>			



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Test Name	Result	Biological Ref. Interval	Unit
Absolute Monocyte Count <i>Method : Calculated</i>	140.60 L	200.00 - 1000.00	/μL
Absolute Basophil Count <i>Method : Calculated</i>	0.00 L	20.00 - 100.00	/μL
Platelet Count <i>Method : Impedance</i>	198.00	150.00 - 410.00	thou/μL
MPV (Mean Platelet Volume) <i>Method : Calculated</i>	8.00	6.80 - 10.90	fL

MICROBIOLOGY

Culture, Urine

Sample : Urine, Random
Method : Manual

Incubation Period 48 hrs/ 37°C ---
Result (Urine) No Growth after 48 hrs of aerobic incubation ---

SEROLOGY

Malaria Antigen Detection (Rapid)

Sample : Whole Blood, EDTA
Method : Immunochromatography

Plasmodium Vivax (PV) Not Detected Not Detected ---
Plasmodium Falciparum (PF) Not Detected Not Detected ---

WIDAL



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Result : Negative

Test Name	Result	Unit	Bio. Ref. Range	Method
SALMONELLA TYPHI 'O'	<1:80	Titre	<1:80	Agglutination
SALMONELLA TYPHI 'H'	<1:80	Titre	<1:80	Agglutination
S.PARATYPHI A 'H'	<1:80	Titre	<1:80	Agglutination
S.PARATYPHI B 'H'	<1:80	Titre	<1:80	Agglutination

Urine Routine & Microscopic Examination

Urine routine examination and microscopy comprises of a set of screening tests that can detect some common diseases like urinary tract infections, kidney disorders, liver problems, diabetes or other metabolic conditions. Physical characteristics (colour and appearance), chemical composition (glucose, protein, ketone, blood, bilirubin and urobilinogen) and microscopic content (pus cells, epithelial cells, RBCs, casts and crystals) are analyzed and reported.

Complete Blood Count (CBC)

CBC comprises of estimation of the cellular components of blood including RBCs, WBCs and Platelets. Mean corpuscular volume (MCV) is a measure of the size of the average RBC, MCH is a measure of the hemoglobin content of the average RBC and MCHC is the hemoglobin concentration per RBC. The red cell distribution width (RDW) is a measure of the degree of variation in RBC size (anisocytosis) and is helpful in distinguishing between some anemias. CBC examination is used as a screening tool to confirm a hematologic disorder, to establish or rule out a diagnosis, to detect an unsuspected hematologic disorder, or to monitor effects of radiation or chemotherapy. Abnormal results may be due to a primary disorder of the cell-producing organs or an underlying disease. Results should be interpreted in conjunction with the patient's clinical picture and appropriate additional testing performed.

Culture, Urine

- * Urine is an ultra-filtrate of blood and is normally Sterile.
- * Urine gets infected via ascending infection by gut flora that is contaminating the perineum.
- * Escherichia coli is the commonest uropathogen
- * Unless there is a catheter or anatomical abnormality, UTI is caused by pure culture of bacteria
- * When present at significant concentration (between 103 cfu/ml to 105 cfu/ml)
- * When a mixture of bacteria are grown on culture after mid stream collection of urine, it signifies contamination of the urine during sample collection and a repeat sample, after taking adequate precautions while collecting sample and without delay transfer into Buffered Boric Acid containing tube in the only solution.
- * In a symptomatic patient, sample should be taken at once, transferred into Buffered Boric Acid tube and sent for culture.
- * Early morning sample is useful for screening purposes from asymptomatic persons.



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Interpretation:

Interpretation of MIC (Minimum Inhibitory Concentration) (Ref CLSI M100Ed33)

1. The MIC is the lowest concentration of drug that inhibits the growth of the bacterial isolate.
2. The antibiotic panels and MIC reference ranges for microorganisms are designed based on CLSI guidelines.
3. The MIC value should be interpreted based on the range tested. The MIC value for one antibiotic cannot be compared with the MIC value of another antibiotic.

Sensitive (S) :	Organism is inhibited by usually achievable concentration of antibiotic with standard dosage.
Intermediate (I) :	Organism is inhibited by higher than normal dosage of antibiotics.
Resistant (R) :	Isolates are NOT inhibited by achievable concentrations of drug with normal dosage schedule.

* No CLSI reference range available.

Note: Culture results should ALWAYS be correlated with clinical findings and prior antibiotic therapy.

Anaerobic and non-cultivable bacteria cannot grow on routine aerobic culture media.

Malaria Antigen Detection (Rapid)

Clinical Significance : Malarial Antigen Rapid card test is used to detect the presence of malarial parasite in blood, specifically 2 species of malarial parasite - Plasmodium vivax and Plasmodium falciparum

Widal

While the definitive diagnosis of typhoid fever depends on the isolation of S typhi from blood, stools, urine or other body fluids, the role of the Widal test had been to increase the index of suspicion for the presence of typhoid fever by demonstrating a positive agglutination during the acute and convalescent period of infection with evidence of a four-fold rise of antibody titre. In many developing countries, including India, the Widal test appears to be the only laboratory means employed in the diagnosis of typhoid fever among suspected patients. As the test suffers from serious cross-reactivity with other infectious agents, it may produce false-positive results, leading to an over-diagnosis of typhoid fever. The Widal test reaction involves the use of bacterial suspensions of S typhi and S paratyphi 'A' and 'B', treated to retain only the 'O' and 'H' antigens. These antigens are employed to detect corresponding antibodies in the serum of a patient suspected of having typhoid fever. The IgM somatic O antibody appears first and represents the initial serologic response in acute typhoid fever, while the IgG flagella H antibody usually develops more slowly but persists for longer.

In an individual with no prior exposure to S typhi infection (either lack of active infection or absence of passive immunisation), a higher than 1:80 or 1:160 titre on an initial single test, usually correlates fairly well with exposure to typhoid fever. However, even these single high value titres in an endemic area where repeated exposures to S typhi may have occurred, do not have any clinical relevance in the absence of a positive isolate of the causative organism.

Researchers from different parts of India have reported that in normally healthy blood donors, the baseline titre for antibodies to "O" and "H" antigens of Salmonella enterica serotype typhi was 1:40 and hence, based on the above results, it could be recommended to use a cutoff level of $\geq 1:80$ for a single antibody test titre. Similarly, baseline titre for antibody to H antigen of Salmonella enterica serotype paratyphi A and paratyphi B was 1:80 and the cutoff level was $\geq 1:160$ for a single antibody test titre.



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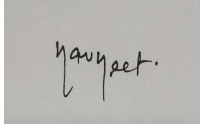
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** End of Report **

Authenticated By

Dr. Navneet

Dr. Nisha
CONSULTANT MICROBIOLOGIST

